

PORO[®] SUPPOSITORY

Active Ingredient:
PORO[®] SUPPOSITORY 500MG
Each suppository contains:
Paracetamol (Acetaminophen) 500mg

PORO[®] SUPPOSITORY 1000MG
Each suppository contains:
Paracetamol (Acetaminophen) 1000mg

Pharmacodynamics:
Mechanism of action
Paracetamol is an aniline derivative with analgesic and antipyretic actions similar to those of aspirin but with no demonstrable anti-inflammatory activity. It does not affect thrombocyte aggregation or bleeding time. Paracetamol is generally well tolerated by patients hypersensitive to acetylsalicylic acid. It produces analgesia by elevation of the pain threshold and antipyresis through action on the hypothalamic heat-regulation centre.

Pharmacokinetics:
Paracetamol is well absorbed by both oral and rectal routes. Peak plasma concentrations occur about 2 to 3 hours after rectal administration. The plasma half life is about 2 ¼ hours and is prolonged in cirrhosis.

Paracetamol is primarily metabolised in the liver by conjugation to glucuronide and sulphate. A small amount (about 3-10% of a therapeutic dose) is metabolised by oxidation and the reactive intermediate metabolite thus formed is bound preferentially to the liver glutathione and excreted as cystein and mercapturic acid conjugates.

Excretion occurs via the kidneys. 2-3% of a therapeutic dose is excreted unchanged; 80-90% as glucuronide and sulphate and a smaller amount as cysteine and mercapturic acid derivatives.

Indication:
PORO[®] SUPPOSITORY 500MG
For the treatment of mild to moderate pain and fever.

PORO[®] SUPPOSITORY 1000MG
For the treatment of mild to moderate pain such as toothache and/or pyrexia.

Paracetamol Suppositories may be especially useful in patients unable to take oral forms of paracetamol, e.g. post-operatively or with nausea and vomiting.

Recommended Dose:
PORO[®] SUPPOSITORY 500MG
Children 10 years – 12 years (30-40kg):
1 suppository every 4 to 6 hours up to a maximum of 4 suppositories in 24 hours.

Adults & Children 12 years and above:
1 -2 suppositories every 4 to 6 hours up to a maximum of 8 suppositories in 24 hours.

Dosages should be based on the child's age and weight. The dose should not be repeated more frequently than every 4 hours. The recommended dose should not be exceeded. Higher doses do not produce any increase in analgesic effect. The product should not be used for more than 3 days, except on the advice of a doctor. Only whole suppositories should be administered – do not break the suppository before administration.

PORO[®] SUPPOSITORY 1000MG
Adults
1 suppository every 4 to 6 hours up to a maximum of 4 suppositories in 24 hours.

The dose should not be repeated more frequently than every 4 hours. The recommended dose should not be exceeded. Higher doses do not produce any increase in analgesic effect. The product should not be used for more than 3 days, except on the advice of a doctor. Only whole suppositories should be administered – do not break suppository before administration.

Route of Administration
Rectal route.

Contraindication:
Hypersensitivity to either paracetamol, soy, peanuts or any of the other ingredients.

Warnings and Precautions:
Paracetamol Suppositories should not be combined with other analgesic medications that contain paracetamol. Paracetamol should be given with care to patients with impaired kidney or liver function.

This preparation contains PARACETAMOL.
Do not take any other paracetamol containing medicines at the same time.

In general, the habitual use of painkillers, especially with combinations of more than one pain killing active ingredient, can lead to permanent kidney damage with the risk of liver failure (analgesic nephropathy).

Allergy alert: Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash. These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away.

Interactions with Other Medicaments:
The absorption of paracetamol is speeded by metaclopramide or domperidone, and absorption is reduced by colestyramine.

The anticoagulant effect of warfarin and other coumarins may be increased by long term regular daily use of paracetamol, with increased risk of bleeding. Occasional doses of paracetamol do not have a significant effect on these anticoagulants.

Enzyme-inducing medicines, such as some antiepileptic drugs (phenytoin, phenobarbital, carbamazepine) have been shown in pharmacokinetic studies to reduce the plasma AUC of paracetamol to approx. 60 %. Other substances with enzyme- inducing properties, e.g. rifampicin are also suspected of causing lowered concentrations of paracetamol. In addition, the risk of liver damage during treatment with maximum recommended doses of paracetamol will be higher in patients being treated with enzyme-inducing agents.

Pregnancy and lactation
A large amount of data on pregnant women indicate neither malformative, nor feto/neonatal toxicity. Epidemiological studies on neurodevelopment in children exposed to paracetamol in utero show inconclusive results. If clinically needed, paracetamol can be used during pregnancy however it should be

Item Code	: 204XXX draft 13 Sep 2021
PM Code	: Ins.P PRB5 & PRB10 (MGH,MMU)
Measurement(mm) ± 2mm	: 260(L) x 110(W)
CURSOR (ATTACHMENT 1)	:
CURSOR (ATTACHMENT 2)	:
Colour of Artwork	: K 100
Source of Artwork	: MAC 1 / CC

L
260mm
±
2mm

W
110mm ± 2mm

used at the lowest effective dose for the shortest possible time and at the lowest possible frequency.

Paracetamol is excreted in breast milk but not in clinically significant amounts. Available published data do not contraindicate breast feeding.

Adverse/Undesirable Effects
Tabulated list of adverse reactions

Common >1/100	<i>Miscellaneous:</i>	Redness of the rectal mucous membranes
Rare <1/1000	<i>General:</i> <i>Skin:</i> <i>Liver:</i> <i>Genitourinary:</i>	Allergic reactions including skin rashes Exanthema, urticaria Liver damage Increase in creatinine (mostly secondary to hepatorenal syndrome)

There have been some reports of blood dyscrasias, including thrombocytopenia and agranulocytosis, with the use of paracetamol-containing products, but the causal relationship has not been established.

Cutaneous hypersensitivity reactions including skin rashes, angioedema, Stevens Johnson Syndrome/ Toxic Epidermal Necrolysis have been reported.

Reporting of suspected adverse reactions

Reporting suspected adverse reactions after authorisation of the medicinal product is important. It allows continued monitoring of the benefit/risk balance of the medicinal product. Healthcare professionals are asked to report any suspected adverse reactions.

Overdose and Treatment

Liver damage is possible in adults who have taken 10g or more of paracetamol. Ingestion of 5g or more of paracetamol may lead to liver damage if the patient has risk factors (see below).

It is considered that excess quantities of a toxic metabolite (usually adequately detoxified by glutathione when normal doses of paracetamol are ingested) become irreversibly bound to liver tissue.

Risk Factors:

If the patient

- Is on long term treatment with carbamazepine, phenobarbitone, phenytoin, primidone, rifampicin, St John's Wort or other drugs that induce liver enzymes.

Or

- Regularly consumes ethanol in excess of recommended amounts

Or

- Is likely to be glutathione deplete e.g. eating disorders, cystic fibrosis, HIV infection, starvation, cachexia.

Symptoms:

Symptoms of paracetamol overdose in the first 24 hours are pallor, nausea, vomiting, anorexia and abdominal pain. Liver damage may become apparent 12 to 48 hours after ingestion. Abnormalities of glucose metabolism and metabolic acidosis may occur. In severe poisoning, hepatic failure may progress to encephalopathy, haemorrhage, hypoglycaemia, cerebral oedema, and death. Acute renal failure with acute tubular necrosis, strongly suggested by loin pain, haematuria and proteinuria, may develop in the absence of severe liver damage. Cardiac arrhythmias and pancreatitis have been reported.

Management:

Immediate treatment is essential in the management of paracetamol overdose. Despite a lack of significant early symptoms, patients should be referred to hospital urgently for immediate medical attention. Symptoms may be limited to nausea or vomiting and may not reflect the severity of overdose or the risk of organ damage. Management should be in accordance with established treatment guidelines, see BNF overdose section.

Treatment with activated charcoal should be considered if the overdose has been taken by mouth within 1 hour. Plasma paracetamol concentration should be measured at 4 hours or later after ingestion (earlier concentrations are unreliable). Treatment with N-acetylcysteine may be used up to 24 hours after ingestion of paracetamol, however, the maximum protective effect is obtained up to 8 hours post-ingestion. The effectiveness of the antidote declines sharply after this time. If required the patient should be given intravenous N-acetylcysteine, in line with the established dosage schedule. If vomiting is not a problem, oral methionine may be a suitable alternative for remote areas, outside hospital.

Management of patients who present with serious hepatic dysfunction beyond 24 hours from ingestion should be discussed with NPIS or a liver unit.

Storage Conditions

Store at temperature below 25°C.

Dosage forms and packaging available

500mg suppository, 5's x 2
1000mg suppository, 5's x 2

Shelf Life:

3 years

Product Description:

A white to off-white spindle shaped suppository.



Manufacturer and Product Registration Holder:
Y.S.P. INDUSTRIES (M) SDN. BHD. (199001001034)
Lot 3, 5 & 7, Jalan P/7, Section 13,
Kawasan Perindustrian Bandar Baru Bangi,
43000 Kajang, Selangor Darul Ehsan, Malaysia.
Ordering Line: 1 800 88 3027
Product Info: 1 800 88 3679

xxxxxx printing code xxxxxx

Date of revision: 13 Sep 2021

Item Code	: 204XXX draft 13 Sep 2021
PM Code	: Ins.P PRB5 & PRB10 (MGH,MMU)
Measurement(mm) ± 2mm	: 260(L) x 110(W)
CURSOR (ATTACHMENT 1)	:
CURSOR (ATTACHMENT 2)	:
Colour of Artwork	: K 100
Source of Artwork	: MAC 1 / CC

L
260mm
±
2mm

W
110mm ± 2mm